

Background

- NICE advise suspecting familial hypercholesterolaemia (FH) as a possible diagnosis in adults with a total cholesterol level > 7.5 mmol/L and/or a personal or family history of premature coronary heart disease (an event before 60 years in an index individual or first-degree relative).
- Primary care records should be systematically searched for people <30 years, with a total cholesterol concentration >7.5 mmol/L and those ≥30 years or older, with a total cholesterol concentration >9.0 mmol/L.
- These are the people who are at highest risk of FH.
- People with a personal or family history of premature coronary heart disease (an event before 60 years in an index individual or first-degree relative), whose total cholesterol is unknown, should be offered a total cholesterol measurement.

Simon Broome Criteria

	Tchol		LDL-C
Adult	>7.5 mmol/L	& or	>4.9 mmol/L
Child	>6.7 mmol/L	& or	>4.0 mmol/L

Simon Broome Criteria AND

Tendon xanthomata or evidence of these in a 1° or 2° relative; **OR**
DNA evidence of an LDL receptor mutation, familial defective apo-B-100 or a PCSK9 mutation.

Definite familial hypercholesterolaemiaSimon Broome Criteria AND

At least one of:

- Personal history of MI/CVD
- Family history of MI/CVD in a 2° relative ≤ 50 yrs or 1° relative ≤ 60 yrs.
- Family history of raised TChol >7.5 mmol/L in adult 1° or 2° relatives or > 6.7 mmol/L in a child/sibling <16 yrs.

Possible familial hypercholesterolaemia

Expect lipids to stabilise within 3 months after normalisation of TSH or with a HbA1C of <60mmol/mol

Consider secondary causes of raised lipids:

Hypothyroidism
Diabetes
Liver disease
Renal disease

Drug induced Cushings
Eating disorders
Anti-psychotic medication
Anti-depression medication
HIV medication
Immunosuppression

Refer to appropriate service if indicated

Routine referral to lipid clinic with

BP, BMI, FBC
Renal function
Urine dipstick (protein)
LFT

TFT
HbA1C
Lipid profile

Note: Primary care is not required to undertake any genetic testing